

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
**for the administration of low-dose diphtheria, tetanus and inactivated
poliomyelitis vaccine (Td/IPV, Revaxis®)
for protection against Tetanus, Diphtheria and Poliomyelitis**

Guideline Summary: Green Book Chapter 30 (Tetanus) and Chapter 15 (Diphtheria)

Assessment and indication

- Tetanus is caused by *Clostridium tetani*, which enters the body through wounds contaminated with soil, dust or manure. It is not transmitted person to person, so protection depends entirely on individual immunisation.
- The objective of the UK programme is a minimum of five doses of tetanus-containing vaccine at appropriate intervals, which is considered to give lifelong protection in most individuals.
- Td/IPV (Revaxis®) is the vaccine of choice for individuals aged 10 years and over who require protection against tetanus, diphtheria or polio.
- The routine adolescent (teenage) booster is given at around 14 years of age (school Year 9), ideally 10 years after the pre-school booster and a minimum of 5 years after it.

Travel indication

- A booster is indicated for travellers to areas where medical attention may not be accessible should a tetanus-prone wound occur, or where tetanus, diphtheria or polio is endemic or epidemic, if the last dose of the relevant antigen was more than 10 years ago. This applies even where five doses have previously been received.
- Some countries require proof of polio vaccination within the previous 12 months for exit or entry. Travellers should be advised to check current NaTHNaC and TravelHealthPro requirements for their destination.

Tetanus-prone wounds

- A reinforcing dose is indicated where the wound is tetanus-prone and the last tetanus-containing dose was more than 10 years ago, or where the immunisation history is incomplete, unknown or uncertain.
- Some wounds also require human tetanus immunoglobulin. Administration of tetanus immunoglobulin is NOT covered by this PGD; these individuals must be referred for same-day medical assessment.

Incomplete or unknown immunisation history

- Where there is no reliable history of previous immunisation, individuals should be assumed to be unimmunised and the full UK schedule followed: a primary course of three doses at monthly intervals, a first booster at least 5 years after the third dose, and a second booster a minimum of 5 and ideally 10 years after the first.
- Individuals arriving in the UK from countries with different schedules, from areas of conflict or from hard-to-reach groups may not be fully immunised against all antigens used in the UK.

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Low-dose diphtheria, tetanus and inactivated poliomyelitis vaccine (Td/IPV, Revaxis®)

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Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training • All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005 • All users must be familiar with Gillick competence and the assessment of consent in children and young people, and with the involvement of a person with parental responsibility

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation of individuals aged 10 years and over against tetanus, diphtheria and poliomyelitis, in accordance with Chapters 15, 26 and 30 of Immunisation Against Infectious Disease (the Green Book). This PGD supports: the routine adolescent booster where this has been missed; completion of a primary course or booster where the immunisation history is incomplete, unknown or out of date; protection for travel in accordance with NaTHNaC recommendations; and a reinforcing dose for a tetanus-prone wound where immunoglobulin is not indicated.
Inclusion criteria	• Individuals aged 10 years and over. • Valid informed consent obtained from the individual, or from a person with parental responsibility where the individual is under 16 and not Gillick competent. • Requires a booster following a primary course of immunisation against diphtheria, tetanus and poliomyelitis (usually offered at 13 to 18 years of age where not already completed); OR

	• Has no history, or an incomplete history, of diphtheria, tetanus or poliomyelitis immunisation; OR • Is travelling to an area where medical attention may not be accessible should a tetanus-prone wound occur, or will be residing in an epidemic or endemic area, and the final dose of the relevant antigen was received more than 10 years ago (this applies even if 5 doses have previously been received); OR • Has a tetanus-prone wound where a reinforcing dose is indicated in accordance with Green Book Chapter 30 Table 30.1 and human tetanus immunoglobulin is NOT indicated.
Exclusion criteria	• Individuals who have not given valid informed consent, or for whom consent from a person with parental responsibility has not been obtained where required. • Aged under 10 years. Refer to the GP or an appropriate immunisation service for age-appropriate vaccine (DTaP/IPV/Hib/HepB or dTaP/IPV). • Confirmed anaphylactic reaction to a previous dose of a diphtheria, tetanus or poliomyelitis containing vaccine, including any conjugate vaccine in which diphtheria or tetanus toxoid is used as the carrier. • Confirmed anaphylactic reaction to any component of the vaccine, including neomycin, streptomycin or polymyxin B. • Acute severe febrile illness (the presence of a minor infection is not a contraindication). • A tetanus-prone wound where human tetanus immunoglobulin is indicated in accordance with Green Book Chapter 30. Refer for same-day medical assessment. • Management of cases or contacts in an outbreak of diphtheria or poliomyelitis. These individuals must be managed by the local Health Protection Team and are outside this PGD. • Pregnant individuals, except where protection is required without delay (see cautions). Refer to the GP or midwife.
Cautions including any relevant action to be taken	• Facilities and trained staff for the management of anaphylaxis must be available at all vaccination premises, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone (Green Book Chapter 8 and Resuscitation Council UK guidance). • Pregnancy: Td/IPV may be given where protection is required without delay, such as following a tetanus-prone wound. However, from week 16 of pregnancy onwards, pertussis-containing vaccine is routinely indicated instead. Under this PGD, refer pregnant individuals to the GP or midwife. • Neurological conditions: the presence of a neurological condition is not a contraindication, but where there is evidence of current neurological deterioration, defer and seek advice so that any change is not incorrectly attributed to the vaccine. • Immunosuppression: the immune response may be reduced. Vaccination is still recommended, and the individual should be advised that protection may be limited. • Syncope (fainting) can occur before or after vaccination, particularly in adolescents. Ensure procedures are in place to avoid injury from faints. • Revaxis® contains approximately 10 micrograms of phenylalanine per 0.5 ml dose. The NSPKU advises this amount is negligible and that individuals with phenylketonuria should

	take up the offer of immunisation. • Bleeding disorders and anticoagulation: see route and method of administration. • If a person attends for a routine booster and has previously received a vaccine following a tetanus-prone wound, establish which vaccine was given. If it was the same vaccine and given after an appropriate interval, the routine booster is not required.
Actions if patient is excluded or declines treatment	Advise on alternative options and how these can be accessed, including the individual's GP practice and, for travel, an appropriate travel health service. Explain the protective effect of the vaccine, the risk of infection and the potential complications of disease. Document any advice given and the decision reached. Inform or refer to the GP as appropriate. Where a tetanus-prone wound requires immunoglobulin or medical assessment, arrange same-day referral and do not delay.

Details of the medicine

Name, form and strength of medicine	Revaxis® (adsorbed diphtheria (low dose), tetanus and inactivated poliomyelitis vaccine), suspension for injection in a pre-filled syringe, 0.5 ml.
Legal category	POM
Black triangle	No
Off-label use	• Primary immunisation in individuals over 10 years of age is off-label but is administered in accordance with Chapters 15, 26 and 30 of the Green Book. • Administration to individuals who have received a diphtheria or tetanus toxoid containing vaccine within the previous 5 years is off-label but is indicated for the management of primary immunisation as described above. • Where a vaccine is administered off-label, inform the individual, parent or carer as part of the consent process that the vaccine is being given outside its product licence but in accordance with national guidance.
Storage	Store at +2°C to +8°C. Do not freeze. Store in the original packaging to protect from light. Vaccine stored outside these conditions must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any decision on continued use.
Route/method of administration	• Administer by intramuscular injection, preferably into the deltoid muscle of the upper arm. • Shake the pre-filled syringe well before administration; the normal appearance is a cloudy white suspension that may sediment during storage. • Visually inspect for foreign particulate matter or any variation from the expected appearance before administration. If present, do not administer and discard in accordance with local procedures. • Where given at the same time as other vaccines, use separate sites, preferably different limbs; if in the same limb, at least 2.5 cm apart. Record the site of each vaccine. • Individuals on stable anticoagulation, including those on warfarin who are up to date with INR testing and whose latest INR was below the upper threshold of their therapeutic range, may be vaccinated intramuscularly using a fine needle (23 gauge or finer), followed by firm pressure without rubbing for at least 2 minutes. Advise on the risk of haematoma. If the intramuscular route is not suitable, give by deep subcutaneous injection.

Dose and frequency of administration	• Single 0.5 ml dose per administration. • Adolescent booster: a single 0.5 ml dose at around 14 years of age, given a minimum of 5 years after the pre-school booster. • Previously unimmunised, or unknown or incomplete history: a primary course of 3 doses with an interval of one month between doses. Where a course is interrupted it should be resumed and not repeated. • First booster: at least 5 years after the third primary dose. Second booster: a minimum of 5 years and ideally 10 years after the first booster, where fewer than 5 documented doses have been given. • Travel: a single booster dose where the last dose was more than 10 years ago. • Tetanus-prone wound: a single reinforcing dose where the last tetanus-containing dose was more than 10 years ago, or in accordance with Green Book Chapter 30 Table 30.1 where the history is incomplete or uncertain.
Duration of treatment	See dose and frequency of administration.
Quantity to be administered and/or supplied	Single 0.5 ml dose per administration.
Disposal	Equipment used for immunisation, including used syringes and any discharged vaccine, must be disposed of in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Drug interactions	May be given at the same time as other vaccines at a separate site. The immunological response may be diminished in individuals receiving immunosuppressive treatment; this is not a reason to withhold vaccination. A detailed list of interactions is available in the SPC.
Adverse effects	Very common: pain, swelling or redness at the injection site; a small painless nodule may form. Common: pyrexia, headache, vertigo, nausea and vomiting. Allergic reactions can occur including urticaria, angioedema, anaphylaxis and shock. Refer to the SPC and PIL for the full list.
Yellow card reporting	Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk . Document any adverse reaction in the individual's record and inform their GP.
Records to be kept	Record the following information: • that valid informed consent was given by the individual, or by a person with parental responsibility, or that a decision was made in the individual's best interests in accordance with the Mental Capacity Act 2005 • name of individual, address, date of birth and GP with whom the individual is registered • name of immuniser • name and brand of vaccine • date of administration • dose, form and route of administration • quantity administered • batch number and expiry date • anatomical site of vaccination • advice given, including advice given if the individual is excluded or declines immunisation • details of any adverse drug reactions and actions taken • that the vaccine was administered via PGD Records should be signed and dated. All records should be clear, legible

Valid from date	Expiry date

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		30/07/2026
Chris Pilkington	Pharmacist GPhC: 2046322		30/07/2026

Change history

Version number	Change details	Date
001	Development and issue of new PGD	30th July 2026
002	Adoption and authorisation blocks corrected: those two signatures belong to the adopting pharmacy's superintendent and professional group signatory, not to Get Real Health, and had been pre-filled in error. Clinical content unchanged.	6th August 2026